Trastuzumab intravenous (IV) · weekly · metastatic

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Trastuzumab intravenous (IV) · weekly · metastatic

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.
- First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. National Cancer Control Programme (NCCP) observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit's approved policy.
- Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00201 v6 · p3 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.14 · Regimen-level drafts; prescriber and oncology-pharmacy verification required